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February 23, 2026

The Honorable Robert F. Kennedy Jr.
Secretary
U.S. Department of Health and Human Services
Mary E. Switzer Building
330 C Street SW
Washington, DC 20201

Submitted electronically via www.regulations.gov

Re: Request for Information; Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Secretary Kennedy:

Kaiser Permanente appreciates the opportunity to comment on HHS' request for information (RFI)[1] soliciting public comment regarding how HHS can accelerate the adoption and use of artificial intelligence (AI) in clinical care (the "HHS Health Sector AI RFI"). Kaiser Permanente[2] is the largest private integrated health care delivery system in the United States, with more than 12.8 million members in nine states and the District of Columbia. Kaiser Permanente's mission is to provide high-quality, affordable health care services and to improve the health of our members and the communities we serve.

AI holds significant potential to improve nearly every aspect of health care. AI can be leveraged to streamline administrative processes and alleviate provider burden while also improving care quality, affordability and patient experience. At the same time, it is critical that AI tools used in health care be safe and effective to build trust among patients and providers. HHS can play an important role in advancing the use of AI by adopting policies that enable health care systems to realize the potential of AI while ensuring tools are safe, effective and provide tangible benefits. Our high priority policy recommendations are summarized below, followed by more detailed comments that provide input on questions and topics addressed in the RFI.

Overall, we recommend that HHS:

- Adopt a national AI framework that is flexible, adaptable and evidence-based.
- Support health care workforce education and training programs to equip providers with resources and training to effectively and responsibly deploy AI tools.
- Enhance federal government and private sector partnerships to support AI innovation, research and development.

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[1] 90 Fed. Reg. 60108 (Dec. 23, 2025).

[2] Kaiser Permanente comprises Kaiser Foundation Health Plan, Inc., one of the nation's largest not-for-profit health plans, and its health plan subsidiaries outside California and Hawaii; the not-for-profit Kaiser Foundation Hospitals, which operates 40 hospitals and more than 600 other clinical facilities; and the Permanente Medical Groups, self-governed physician group practices that exclusively contract with Kaiser Foundation Health Plan and its health plan subsidiaries to meet the health needs of Kaiser Permanente's members.

REGULATION

HHS states in the RFI that the Department seeks to "establish a regulatory posture on AI that is well understood, predictable and proportionate to any risks presented to enable rapid innovation while protecting patients and the confidentiality of their identifiable health information, and maintaining public trust."[3] Kaiser Permanente appreciates these efforts and offers the following feedback and recommendations regarding how regulation impacts AI adoption and use in clinical care.

Complex Regulatory and Enforcement Environment

Oversight is important to assure patient safety and care quality, however, the emerging complex regulatory environment with layers of inconsistent guidance and requirements from industry, states and federal agencies hampers health care systems' ability to adopt AI tools and diminishes the efficiencies and cost savings that use of the tools could bring to clinical care.

We therefore recommend that HHS work with stakeholders to establish a federal framework for governing AI used in health care that preempts conflicting state laws. This approach will enable health systems to focus their time and resources on innovating and deploying tools rather than tracking and complying with the proliferation of inconsistent or duplicative requirements. Adoption of a federal AI regulatory framework will also support private sector activities (e.g., accreditation, certification, industry-driven testing and credentialing) by providing a consistent structure to build upon and align with.

The federal AI regulatory framework should be flexible, adaptable and evidence-based. It should provide an overarching structure that is risk-based, consistent with nationally recognized AI risk management frameworks, such as the National Institute of Standards and Technology (NIST) AI Risk Management Framework and the International Organization for Standardization/International Electrotechnical Commission (ISO/IEC) AI Guidance on Risk Management. The framework should also establish guardrails to ensure patient safety while being flexible and adaptable to keep pace with rapidly evolving technology. It should focus on monitoring outcomes rather than imposing prescriptive governance requirements and be scalable by organizations of all sizes and levels of sophistication. This will allow all patients to benefit from innovative technologies while also being protected from potential harm.

We recommend that the federal regulatory framework exclude from classification as "developers" those health care entities that develop AI tools for their own internal use and do not make the tools available to others. This approach is consistent with other federal medical software device policies and promotes access to innovative technologies while reducing costs and administrative burden. Health care entities that develop tools carefully tailor them to work within our clinical workflows based on our patient populations, resulting in tools that are more efficient and clinically effective. This approach also reduces risks associated with AI tools developed by third parties, such as potential misuse, unwanted bias, or data inconsistencies or vulnerabilities. Health care entities that self-develop tools are ultimately responsible for how the tool performs, and many of the

[3] 90 Fed. Reg. at 60109.

requirements applicable to developers (e.g., transparency reporting requirements) would not make sense to apply to entities that both develop and deploy the tool internally.

Lastly, it is critical that a federal AI regulatory framework be coordinated across federal agencies and across states to avoid duplicative and/or conflicting guidance and enforcement. As noted above, proliferation of differing state-specific requirements will increase administrative burden and create compliance risks that will discourage AI adoption. Similarly, multiple regulators overseeing the same requirements can create confusion and lead to differences in interpretation and enforcement. We recommend that AI oversight be coordinated among expert federal agencies, such as ASTP/ONC and FDA, well versed in emerging technology and associated risks and benefits.

Evidence

Many health care organizations have been slow to adopt AI tools due to the lack of evidence demonstrating safety and effectiveness. This is compounded by the lack of nationally established standards to measure key functional aspects of AI tools such as, accuracy, explainability and reliability. We recommend that HHS partner with the private sector and standards setting organizations to develop standards that measure these functional aspects of AI tools and systems to enable health systems to accurately evaluate performance across varying clinical settings and applications.

We also recommend that HHS seek out private sector partnership opportunities to support the continued development and use of AI in clinical care. For example, rigorous research studies, such as health system-embedded studies and other hybrid implementation-effectiveness studies, should be leveraged to create a robust body of data and evidence that demonstrate how AI tools perform across patient populations in real-world settings. This approach would promote AI adoption by allowing a wide range of health care entities to efficiently evaluate the safety and efficacy of AI tools. Establishing a voluntary post-implementation surveillance program would also promote AI adoption by enabling health care entities to share data to inform best practices and highlight potential issues in a more efficient and cost-effective manner.

Trust

Lack of trust in AI tools from patients and providers is a fundamental barrier that often inhibits AI tool adoption and efforts to promote trust should be designed and advanced with both patients and providers in mind. Concerns over the effectiveness of the tools, impacts to care quality, patient experience and the privacy and security of health care data all contribute to this lack of trust. Some providers are also hesitant to use AI-enabled tools because they do not understand how the tools were developed, how they work, or how to use them in practice.

To address these concerns, we recommend that HHS work with stakeholders to develop and implement a set of minimum documentation requirements for developers of AI-enabled health care tools to disclose to the entities that will use the tool. This should include sufficient information to help providers understand how the tool was developed, including what data was used to train the tool, and how it is intended to function. Such transparency is particularly important in health care to help providers determine if a tool is appropriate for their needs. For example, a risk stratification tool that was trained with Medicare beneficiary data may not be appropriate for use in pediatric practices.

We also recommend that HHS support health care workforce education and training programs to equip providers with the resources and training to effectively and responsibly deploy AI tools. HHS should publish implementation toolkits along with best practices to guide health care organizations, along with sponsoring AI resource centers such as the Kaiser Permanente Augmented Intelligence in Medicine and Healthcare Initiative (AIM-HI) Coordinating Center[4] that demonstrate the value clinical AI tools can bring to providers and patients across diverse care settings.

Lastly, we recommend that HHS develop and launch education campaigns for consumers, particularly those enrolled in public programs such as Medicaid and Medicare. These education campaigns should explain how AI can be used to improve care and help consumers differentiate between patient wellness devices and applications and AI-enabled tools used by providers, including the differences in required privacy and security protections.

Privacy and Security of Patient Data

Patient health data is essential to developing, validating and deploying AI tools used in clinical care, yet patient privacy is consistently cited as a barrier to adoption and use. While the Health Insurance Portability and Accountability Act (HIPAA) Privacy and Security Rules provide a robust framework that protects information handled by covered entities and business associates, most privacy risks related to AI in health care occur outside HIPAA's scope—among consumer applications, data brokers and other entities that are neither covered entities nor business associates.

We recommend that HHS issue guidance that describes existing privacy and security protections and how they apply to the use of AI data in health care. This guidance should highlight any gaps or opportunities, such as consumer-directed tools and applications, so that any additional protections align with and build upon the existing regulatory framework. Specifically, guidance should:

- Clarify how the de-identification standards in 45 CFR 164.514 apply to AI model training and validation, including the use of limited datasets and synthetic data;
- Outline expectations for business associate agreements (BAAs) when vendors or service providers host, fine-tune or deploy AI models using protected health information; and
- Provide examples of compliant implementation of the HIPAA Security Rule for AI systems, including encryption, access controls and use of privacy-enhancing technologies, such as secure enclaves and federated learning.

[4] Greene, Jan, (Dec. 18, 2023) *Kaiser Permanente coordinates real-world demonstrations of AI, machine learning in health care*, Kaiser Permanente Division of Research, available at https://divisionofresearch.kaiserpermanente.org/kaiser-permanente-ai-machine-learning-in-health-care/

This guidance would reduce uncertainty for covered entities, strengthen compliance consistency across the health care sector, and avoid the need for potentially duplicative AI-specific privacy regulations.

We also recommend that HHS work with stakeholders to develop a federal approach to closing the privacy gap that occurs outside of HIPAA's scope. This approach should establish baseline privacy and security requirements for non-HIPAA actors that collect or infer health-related data while preserving the statute's established framework for regulated health care organizations.

Accountability

A lack of clear accountability structures leads to uncertainty about who is responsible for AI-related errors or negative outcomes which discourages adoption, especially for higher-risk applications such as generative tools used in clinical applications. We recommend that HHS issue interpretive guidance that defines accountability for outcomes, distinguishing between developer and deployer responsibilities. Deployers should be responsible for their use of the AI tool while developers should be accountable for data quality (including any third-party data sources used for development or validation), model and system performance, and design flaws.

Promote Value-Based and Integrated Care

Integrated care models, such as Kaiser Permanente, facilitate coordinated care that improves patient outcomes[5] in a cost-effective manner. Health condition-specific digital tools and programs may help patients better understand and manage their health conditions. However, there is a risk that these programs and tools may fragment care delivery by dividing up care management activities and accountabilities across various entities without a corresponding mechanism to share data and coordinate care, which can lead to poorer outcomes, especially for individuals with multiple chronic health conditions.

Additionally, current payment models do not create incentives for health systems to leverage high-quality and clinically appropriate virtual care and AI tools to augment traditional care delivery modes. We encourage HHS to explore reimbursement and program design approaches that support integration of these tools and programs into existing health system clinical workflows and allow flexibility across care settings with aligned incentives based on the total cost of care.

RESEARCH AND DEVELOPMENT

We appreciate efforts by HHS to investigate ways to invest in research and development to integrate AI in care delivery and create new, long-term market opportunities to improve the health and wellbeing of all Americans.

Published Findings on Impact of Adopted AI Tools and Use in Clinical Care

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[5] Cooke, K., Kaiser Permanente: Highlights of an integrated care delivery system. Hospital Administration and Medical Practices (Sept. 2024), available at https://doi.org/10.54844/hamp.2024.0046

Investments in research and development can provide patients with access to life-changing advancements in care delivery that improve quality of care and clinical outcomes. At Kaiser Permanente, we are working to advance understanding of how AI tools perform in actual health care delivery settings with appropriate guardrails and monitoring. We have published several studies that demonstrate the effectiveness of these tools, including:

- Advance Alert Monitor: An innovative AI system that is estimated to save 500 lives a year[6] in our Northern California hospitals by using predictive analytics to identify at-risk hospitalized patients and prevent deterioration and reduce adverse outcomes before they occur.[7]
- Transition Support Level Score: An AI system that uses predictive analytics to identify patients that would benefit from a care coordination intervention after hospital discharge which resulted in reduced hospital readmission rates.[8]
- Desktop Medicine Program: An AI system that uses natural language processing algorithms to optimize patient-physician email communication[9] and reduce provider burden.
- Ambient AI Scribes: AI system that uses machine learning to facilitate scribe-like capabilities in real time to improve physician-patient interaction and improve administrative efficiency.[10] This tool has supported over 2.5 million visits and our physicians experienced a reduction in clinical documentation workload of more than 15,700 hours, equivalent to 1,794 working days, over a year of use compared to non-users.[11]
- Kaiser Permanente Intelligent Navigator: AI system that augments the patient portal appointment booking workflow to enhance care navigation and improve the patient experience by using natural language processing to detect high-acuity cases and recommend scheduling options for timely and appropriate care. Patients using the system reported improved satisfaction and ease of navigation.[12]

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[6] Martinez, et al., The Kaiser Permanente Northern California Advance Alert Monitor Program: An Automated Early Warning System for Adults at Risk for In-Hospital Clinical Deterioration, *Joint Commission Journal on Quality and Patient Safety* (Aug. 2022), available at https://www.sciencedirect.com/science/article/pii/S1553725022001106.
[7] Escobar, G et al., Automated Identification of Adults at Risk for In-Hospital Clinical Deterioration. *The New England Journal of Medicine* (Nov. 2020), available at https://www.nejm.org/doi/full/10.1056/NEJMsa2001090
[8] Marafino, B. et al., Evaluation of an intervention targeted with predictive analytics to prevent readmissions in an integrated health system: observational study. *BMJ*,(Aug 2021), available at https://www.bmj.com/content/374/bmj.n1747
[9] Liu VX, et al., Content of Patient Electronic Messages to Physicians in a Large Integrated System. *JAMA Network Open*,*(*April 2024), available at https://jamanetwork.com/journals/jamanetworkopen/fullarticle/2817150?resultClick=3
[10] Tierney, et al., Ambient Artificial Intelligence Scribes to Alleviate the Burden of Clinical Documentation. *NEJM Catalyst* (Feb. 2024), available at https://catalyst.nejm.org/doi/full/10.1056/CAT.23.0404
[11] Tierney, et al., Ambient Artificial Intelligence Scribes: Learnings after 1 Year and Over 2.5 Million Uses, *NEJM Catal Innov Care Deliv* (Mar. 2025) available at https://catalyst.nejm.org/doi/full/10.1056/CAT.25.0040 .
[12] Nguyen, D., Lee, S., Synghal, R. et al., Digital transformation with clinical alerts and personalized care systems in an integrated value based model. *npj Digit. Med.* (July 2025), available at https://doi.org/10.1038/s41746-025-01838-1

Consumer Directed Tools

Despite concerns related to AI, patients and caregivers are increasingly using AI-enabled tools such as health care apps and websites to help them manage their health and access care.[13]

The FDA recently clarified that many low-risk software tools[14] and consumer wearables[15] fall outside of medical device regulation. This guidance is intended to focus regulatory oversight on products that present meaningful risks to patient safety, while allowing lower-risk consumer technologies to evolve without unnecessary regulatory burden. Under the new guidance, FDA may consider products that use non-invasive sensing to estimate, infer or output physiologic parameters (such as heart rate variability, blood glucose, oxygen saturation or blood pressure) to be non-device general wellness products when the outputs are intended solely for wellness uses.

Wellness devices are popular with consumers and wearable technology is becoming much more capable and user-friendly. However, despite their popularity, consumer wearables continue to fall short in data accuracy and consistency, especially when compared to medical devices. Because sensors used in wellness devices can vary, two similar wearables might show different readings for the same health metric. Such inconsistency limits their utility in health care where even small errors can create risk, and unreliable data can cause unnecessary worry or, conversely, a false sense of security among users. Nevertheless, wellness devices and applications can be useful tools to promote healthy behaviors and to improve population well-being.

HHS should prioritize research to better understand how patients are using consumer directed tools, including wellness devices. This research should be used to determine the benefits associated with use, along with any guardrails that should be put in place to ensure that the tools add value and do not create confusion for patients and add administrative burden, complexity and risk for providers. We caution against pilots or programs that directly incorporate data from unregulated consumer directed tools into the patient's medical record due to data accuracy concerns and variability in how the various tools capture and record data. However, we encourage HHS to establish best practices to guide wellness tool developers and educate patients and providers about the utility of wellness devices and applications to promote appropriate use.

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Kaiser Permanente appreciates HHS' consideration of our comments, and we look forward to continued collaboration. Please feel free to contact me at erin.e.richardson@kp.org or Megan Lane at Megan.A.Lane@kp.org with questions or if we can provide additional information.

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[13] Sparks, Grace, Montalvo III, Julian, et al., *KFF Health Tracking Poll: Public Use and Trust in Health Care Apps and Websites*, KFF (Oct, 24, 2025), available at https://www.kff.org/public-opinion/kff-health-tracking-poll-public-use-and-trust-in-health-care-apps-and-websites/

[14] U.S. Food & Drug Administration (Jan., 2026) *Clinical Decision Support Software: Guidance for Industry and Food and Drug Administration Staff*, Center for Devices and Radiological Health, Center for Biologics Evaluation and Research, Center for Drug Evaluation and Research, available at https://www.fda.gov/regulatory-information/search-fda-guidance-documents/clinical-decision-support-software

[15] U.S. Food & Drug Administration (Jan., 2026) *General Wellness: Policy for Low Risk Devices*, Center for Devices and Radiological Health, available at https://www.fda.gov/regulatory-information/search-fda-guidance-documents/general-wellness-policy-low-risk-devices

Sincerely,

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The Permanente Federation